

للأشعة و السونار و المفراس و الرنين المغناطيسي

Patient Name: HUSSAM SALAH	Age: 38 Year(s), 11 Month(s)
Study Date: 02/09/2026 09:51:21 PM	Ref. Phy.:

03 September 2026

CT SCAN OF THE ABDOMEN AND PELVIS

With IV contrast

FINDINGS

- Two enhancing polypoidal soft tissue masses are noted within the urinary bladder adjacent to the vesicoureteric junctions. Both lesions demonstrate relatively early enhancement during the arterial phase. The left-sided lesion measures approximately 18 × 10 mm and demonstrates a relatively narrow pedicle measuring no more than approximately 5 mm. The right-sided lesion measures approximately 13 × 8 mm and demonstrates a broader mural attachment measuring up to approximately 12 mm. On correlation with Doppler ultrasound, both lesions demonstrate internal vascularity with identifiable feeding vessels extending through their respective pedicles/attachments. No associated ureteric or PCS dilatation. No obvious extension into either ureter. No perivesical fat stranding or CT evidence of extravesical extension.
- Few small hypodense hepatic lesions are noted, the largest measuring approximately 7.7 mm, without obvious post-contrast enhancement. These lesions are too small to characterize confidently.
- Left kidney is in normal position and slightly small in size, measuring approximately 90 × 45 × 42 mm, with mild decrease in parenchymal thickness and lobulated contour. Normal contrast excretion. No focal lesion. No PCS dilatation. No stone. Normal ureter.
- Right kidney is normal in position and parenchymal thickness, measuring approximately 118 × 70 mm. Normal contrast excretion. No focal lesion. No PCS dilatation. No stone. Normal ureter.
- Adrenal glands are normal in size and appearance.

Liver is normal in size with homogeneous enhancement, apart from the described small hypodense lesions. No dilation of the biliary tree.

Gallbladder is unremarkable.

Spleen is normal in size, homogeneous density, no focal lesion.

Pancreas is normal in size, homogeneous density, no focal lesion.

No significant lymphadenopathy.

No ascites.

No free air.

No destructive bony lesion detected.

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IMPRESSION:



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- Two vascular enhancing polypoidal urinary bladder masses. Multifocal urothelial neoplasms are the primary diagnostic consideration. Benign vascular or inflammatory polypoidal lesions are less likely differential considerations. No associated obstructive uropathy, obvious ureteric extension, significant regional lymphadenopathy, or CT evidence of extravesical extension.
- Few subcentimeter hypodense hepatic lesions, too small to characterize. Small benign cystic lesions are a consideration; however, in the setting of suspected urinary bladder neoplasm, metastatic deposits cannot be confidently excluded on the current examination.

Recommendations

- Cystoscopic evaluation with biopsy/transurethral resection and histopathological assessment is recommended for definitive characterization of the urinary bladder lesions.
- Contrast-enhanced MRI of the liver may be considered for further characterization of the small hepatic lesions, if clinically indicated. Follow-up imaging may alternatively be considered to document stability.

Dr.Ali Basim

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